

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 72 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: March 9, 2017 | Provider: Eleanor Park, MD — Internal Medicine | Doc ID: RFH-2017-0309-FU

Chronic Disease Follow-up

Diabetes Intensification & Newly Recognized CKD Stage 3a

Subjective

72-year-old for diabetes and kidney review. Fasting glucose creeping into 160-180s; some increased thirst. No hypoglycemia. AF and prostate cancer stable. Mild bilateral knee stiffness with activity (new complaint).

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
128/76	70 irreg	15	36.6 C	97% RA	95.8 kg	31.2

Laboratory Review

Test	Result	Units	Reference Range	Flag
Hemoglobin A1c	8.2	%	< 7.5 (individualized)	H
Fasting glucose	172	mg/dL	70 - 130	H
Creatinine	1.3	mg/dL	0.7 - 1.3	
eGFR (CKD-EPI)	55	mL/min/1.73 m2	> 60	L
Urine albumin/creatinine	112	mg/g	< 30	H
Potassium	4.7	mmol/L	3.5 - 5.1	
LDL cholesterol	62	mg/dL	< 70	

Assessment

- T2DM, suboptimal control (A1c 8.2%) with diabetic kidney disease (CKD G3a, ACR 112 = A3).
- CKD stage 3a — confirmed, attributable to diabetes + HTN. eGFR safe for metformin (>45) with monitoring.
- AF — stable on apixaban (note CKD affects dosing thresholds).
- Bilateral knee discomfort — likely early osteoarthritis; imaging if persistent.

Plan

- Add empagliflozin 10 mg PO daily — glycemic, cardiac, and renal protective; counseled on genitourinary infection risk and sick-day rules. (Empagliflozin chosen for CKD/HF benefit.)
- Continue metformin 1000 mg BID (eGFR >45).
- Continue lisinopril for albuminuria.
- Ophthalmology referral for diabetic retinal screening (overdue).
- Recheck A1c, BMP, ACR in 3 months. Knee X-rays if symptoms persist.

Medication Changes

Medication	Dose	Route	Frequency	Indication
Empagliflozin	10 mg	PO	Once daily	T2DM / cardiorenal protection
Metformin	1000 mg	PO	Twice daily	T2DM
Lisinopril	20 mg	PO	Once daily	HTN / albuminuria